



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Restrictive practices

Reference #: FSFH-MENH-POL-0019

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Services	Medical, Nursing, Allied Health, Security Incident Management Services and SMHS security services
EXCLUSION: Cockburn Health		

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1. Introduction

The FSHFG Mental Health Service is committed to providing care in the least restrictive manner possible, however it is acknowledged that there may be occasions when restrictive practices e.g., restraint and/or seclusion are required to protect the safety of patients, staff and visitors. This document supports staff to ensure that such interventions are performed in accordance with the principles of recovery focussed and trauma informed care and in compliance with legislation relating to restraint and seclusion.

2. Terminology

Bodily restraint	The physical or mechanical restraint of a person who is being provided with treatment or care at an authorised hospital ⁶ .
De-escalation	The use of techniques (including verbal and non-verbal communication skills) aimed at defusing anger and averting aggression.
Mechanical restraint	The restraint of a person by application of a device (for example belt, harness, manacle, sheet or strap) to the person's body to restrict their movement.

Nominated person	Person nominated by the patient to assist them to ensure that their rights and interests are upheld according to section 273 (1) of the MHA 2014. The nominated person is whomever the patient feels should receive information and who can help.
Notifiable Incident	Incident classified as notifiable in section 525 of the MHA 2014 in respect of a person referred to in section 524 which must be reported to the Chief Psychiatrist (CP).
Personal support person (PSP)	Person who is supporting someone who is experiencing mental illness. May be any of the following: close family member, carer, nominated person, the parent or guardian of a child and any guardian or enduring guardian adult. ⁶
Physical restraint	The application of bodily force to a person's body to restrict the person's movement; physical restriction of a person to prevent person from harming him/herself or endangering others or to facilitate the provision of essential medical treatment. ¹
Recovery focused care	Care delivery which includes the principles of maximising choice; supporting positive risk taking; promoting dignity; complying with legislation and promoting safety. ¹
Trauma informed care	Where all staff providing services and care are alert to the possibility of trauma in the lives of all patients regardless of whether it is known to exist in individual cases. Key principles of trauma informed care are safety, trustworthiness, choice, collaboration and empowerment. ²
Reasonable force	In the context of restraint situations, refers to a level of force which must be: • absolutely necessary – no other safe alternative • of a degree of force that is required under the circumstances • justified • reasonable in accordance with Australian law.⁵
Restraint	The restriction of an individual's freedom of movement by physical or mechanical means. ¹
Restrictive	Interventions restricting the rights or freedom of movement of

practices	a person. ³ Includes but not limited to restraint and seclusion.
Seclusion	The confinement of a person who is being provided with treatment or care at an authorised hospital by leaving the person alone in a room or area from which it is not within their control to leave: Also applies to situations where the person agrees to or requests confinement but cannot leave of their own accord. ⁴
Timeout	Supporting a patient to remove themselves voluntarily and temporarily to a more suitable environment e.g. low stimuli, calming. The patient agrees to go into a 'time out' room that is not locked and has the ability to leave as desired.
Treatment Support and Discharge Plan (TSDP)	A written document developed collaboratively with the patient and (with consent) their family/significant other and/or PSP, which identifies who is involved in the consumer's care. It is an agreed plan to meet their needs and strategies to ensure safety and mitigate risks. Refer to MHA Section 186.

3. Policy

- Restrictive practices are not therapies and will only be used as a safety measure of last resort and limited to those situations⁸ where all non-pharmacological interventions (de-escalation, non-physical diversion techniques), pro re nata (PRN) medication and all other avenues have been explored and have failed to reduce a situation.⁸
- Clinical care will focus on the elimination of restrictive practices in the treatment setting.
- It will be presumed that every patient has been exposed to trauma in the past and as such all restrictive practices will be in accordance with trauma informed principles.²
- Individual patient needs will be recognised, including sensitivity to cultural, spiritual, language and gender concerns.
- Restraint and/or seclusion must never be used:
 - to overcome a lack of supervision⁸
 - as a punitive measure
 - to inflict pain, suffering or humiliation, or establish dominance⁹

- if bodily restraint on the patient poses a significant risk to their physical health
- where there is inadequate staffing, or the setting is inappropriate.
- The treating psychiatrist or duty Psychiatrist will take an active leadership role in consultation with the patient, the PSP and the MDT in facilitating strategies to reduce seclusion and restraint. Refer to the FSFHG “Anticipating and reducing the risk of unpredictable behaviours” policy. Individualised strategies will be documented in the mental health patient safety plan.
- The treating team/duty medical officer will at the earliest possible time:
 - attend an inpatient unit when there is evidence of escalating patient risk not responding to treatment.
 - take an active decision-making role in the seclusion and restraint process.
- Restrictive practices must be implemented in a manner that is safe, respectful, compassionate, for the minimum amount of time⁸ and only using force that is considered reasonable.
- Restrictive practices will be applied as per the legislative requirements of the MHA 2014 Part 14 Division 5 – Seclusion and Part 14 Division 6 – Bodily restraint.^{10,11}
- In the event that physical restraint is deemed necessary, staff will activate a Code Black as per hospital emergency procedures.

Mechanical restraint will not be used in authorised areas of the FSFHG Mental Health Service.

In extreme circumstances the requirement for the use of mechanical restraint for an individual patient will be escalated to the Head of Service and the Chief Psychiatrist will be consulted.

The MHA 2014 Part 14 Division 6 s227 definition of mechanical restraint does not include the appropriate use of a medical or surgical appliances in the treatment of a physical illness or injury or the appropriate use of furniture that restricts a person’s capacity to get off the furniture (for example a chair fitted with a table across the arms). Refer to the FH” Fixed Tray Table” policy.

- At no time will staff put themselves at risk if an individual is armed with a weapon or the risk to staff and the patient is deemed too high for staff to safely manage an incident. Staff will call a Code Black and the police will be contacted immediately on ‘000’.

- All staff involved in the restraint and/or seclusion of a patient must have completed Aggression Prevention and Intervention (API) training specific to their role and have a sound knowledge of relevant legislation and preventative patient care including de-escalation and/or conflict resolution.
- Ongoing knowledge and skills training will be undertaken in the ward environment in the form of education or simulated Code Black drills.
- Peer workers or persons with lived experience will contribute to staff knowledge wherever possible.
- Aboriginal mental health Liaison will be consulted and utilised where appropriate.
- The following are Notifiable Incidents and must be reported to the Chief Psychiatrist via Datix Clinical Incident Management System (CIMS):
 - assault and/or aggression
 - any physical, verbal or sexual behaviour leading to restraint and/or seclusion.
 - any harm sustained from or during the restraint and/or seclusion process

4. Personal support persons

- Where possible PSPs must be included in all treatment decisions regarding restraint unless the restraint or seclusion is required in an emergency to prevent imminent harm to the patient or others.
 - On admission, staff will obtain the patient's consent to involve their PSP in their care and provide them with relevant information.
 - The patient's PSP will be advised of any restraint and/or seclusion event. This will be documented in the patient's medical record.
 - Consideration will always be given to the best interests of the patient when making the decision to inform the PSP.
 - Staff will collaborate with parents when managing violence and aggression in patients under 18 years.
 - PSPs will be made aware of the Health Service's patient feedback/complaints process should they have any concerns to be addressed.

5. Pharmacological management of agitation and arousal

Agitation and arousal medications may be required for a patient who is aroused, agitated, overactive, aggressive or making serious threats or gestures towards others

or themselves, or being destructive to their surroundings, where other non-pharmacological interventions have failed to contain the behaviour. Refer to the “Medication Management of Agitation and Arousal in Mental Health Inpatients” policy & procedure and Zuclopenthixol acetate (Clopixol acuphase) Specialised Drug Guideline.

6. Security staff role

- Restraint will always be initiated by clinical staff, security staff trained in the application of restraint may be requested to assist under the direction of the Lead Clinician as per the MHA 2014 section 172.
- When Security staff are in attendance the lead clinician always:
 - remains responsible for the patient.
 - makes decisions involving the restrictive practice.
 - guides Security and informs of the rationale for intervention whenever possible.
- Whenever possible the lead clinician will inform Security staff of the rationale for the request to intervene.
- Clinical staff will provide a handover in the iSoBAR format to the Security team on arrival including any medical conditions and/or considerations that may impact the safety of the patient during restraint.
- Security staff will complete their specific incident reporting requirements.
- Regular communication huddles with SIMS will be used for complex care patients to plan treatment and intervention.

7. Physical restraint

- The act of restraint can cause acute distress at the time and may also have long term effects including traumatic memories and symptoms of post-traumatic stress disorder.
- The potential for negative experiences and active re-traumatisation when restraint is used on patients with a history of trauma or abuse is high. It can lead to increased suffering and staff may also be at increased risk if these interventions are used.
- Physical restraint:
 - may be necessary to enable the medication administration for behavioural disturbance or where the patient requires urgent medical and/or psychiatric care and is unable to consent.

- will be in the least restrictive form following a comprehensive assessment of the patient.
- must be used as a temporary measure for the shortest possible time.
- risks to staff involved should always be considered prior to initiating the restraint.
- Physical restraint does not include:
 - the provision of physical support or assistance to carry out activities of daily living
 - equipment that is used to aid care delivery e.g. lap tray used to assist with meals
 - medical or surgical appliances for the proper treatment of physical disease or injury e.g. traction
 - physically redirecting a disoriented person using minimal restraint
- If a non-registered companion/patient care assistant/assistant in nursing or security special is observing the patient it remains the responsibility of the allocated registered nurse or mental health practitioner to perform and document restraint interventions and restraint related patient observations.

7.1. Patient risk factors

- There is no absolute safe restraint intervention. A patient's response to restraint will depend on their individual characteristics, duration of restraint, forcefulness of the hold and a range of other factors including underlying disease, levels of stress and the attitudes projected by all involved. Refer to [Appendix 1](#) Patient Risk Factors for Physical Restraint.
- The Multidisciplinary team (MDT) will discuss individual patient risk factors where possible when planning restraint.

7.2. Team response

- Physical restraint requires a swift coordinated team approach of appropriately trained staff with clearly defined roles. Refer to [Appendix 2](#) Physical restraint: Team roles and responsibilities for roles and responsibilities.
 - An Incident Coordinator will be identified and take responsibility for leading the team through the restraint:
 - Only one member of staff is to communicate with the patient at any one time to prevent confusion and excess stimulation.

7.3. Physical restraint positions

- A patient may be physically restrained in a number of positions depending on the level of aggression, physiological concerns and individual risk assessment.

- The use of restrictive interventions creates significant risks for the person being restrained. Each restraint position has its own risks for harm to the patient and/or staff. Refer to [Appendix 3](#) Physical Restraint Positions and Specific Risks.

Prone (face down) restraint should be limited to the minimum time necessary to change position i.e. less than 3 minutes

During physical restraint under no circumstances should direct pressure be applied to the neck, trachea, thorax, abdomen, back or pelvic areas.

The patient's airway and head must always be protected and monitored to reduce the risk of asphyxiation or head trauma.

- Safety principles to be observed for all positions:
 - never occlude/cover the nose and mouth⁸ unless medically approved e.g. oxygen mask.
 - do not use restraint in a way that interferes with the patient's ability to communicate⁸
 - never pull on, rotate or flex the neck in any direction.

Never apply head locks, choke holds, neck holds or similar.

- avoid hyperflexion of the trunk in standing or seated restraints
- continually assess the airway and protect the head if necessary.
- when moving a patient from a standing position to prone or supine on the floor, support the patient during the descent and protect the patient's head from any impact.
- consider the possibility of a history of sexual assault/trauma and/or pregnancy.

If concerns regarding a patient's airway are raised, the restraint should stop immediately.

If a patient physically deteriorates or reports, they cannot breathe a physiological patient examination must occur immediately and identified issues addressed.

Activate a MET call in the event of a medical emergency.

7.4. Patient observation and management

- All patients require careful management and monitoring of their physical health, both during and after restraint

- While in restraint a mental health practitioner or registered or enrolled nurse must be with the patient at all times providing ongoing care⁶ and ensuring patient dignity. Restraint observations may include:
 - level of consciousness
 - respiration rate
 - airway patency
 - chest movement
 - position (restraint not causing pain)
 - behaviour and mood
 - skin integrity noting any discolouration, bruising or swelling
 - temperature
 - level of patient distress
- Physiological observations will be documented using the appropriate Observation and Response Chart (AORC). Refer to the FSFHG “Observation and Response Chart” policy and procedure
- Efforts to re-engage the patient and re-establish a therapeutic rapport following physical restraint will be implemented and documented in the patient’s medical record using the Post Restraint Interview Form.

7.5. MHA 2014 obligations

- A copy of all forms completed must be provided to the CP. Refer to Table 1 MHA 2014 obligations: Restraint for details.

Table 1. MHA 2014 obligations: Restraint

MHA 2014 Form	MHA 2014 obligations
10A	Record of oral authorisation of bodily restraint Physical restraints can be authorised verbally by: • a medical practitioner • mental health practitioner or the person in charge of the ward (refers to NUM, HOOT Manager or shift coordinator) If the oral authorisation is not confirmed using a Form 10B within 30 minutes of the person being restrained the person cannot continue to be restrained.
10B	Written bodily restraint order A Form 10B is required to confirm a Form 10A when the restraint

	continues >30 minutes. A medical practitioner, a mental health practitioner or person in charge of a ward may complete a written authorisation. If the restraint is used with the intention of placing a patient into seclusion, it is not necessarily to make a Restraint Order. Where a patient is restrained and then a decision is made that they require seclusion both Restraint and Seclusion Orders must be made.
10C	Record of information Medical Practitioner and Treating Psychiatrist of restraint order When a mental health practitioner or the nurse in charge of the ward gives an oral authorisation of bodily restraint or makes a bodily restraint order: • a medical practitioner must be informed of the restraint or that the patient has been restrained as soon as practicable, to enable an examination by a medical practitioner within 30 minutes of the restraint commencing (MHA2014 s.230 & s.233) • the treating psychiatrist must be informed of the restraint as soon as practicable, and within 30 minutes of the restraint beginning (MHA2014 s.233)
10D	Record of observations made of restrained person. To ensure that the patient's physical and mental health is monitored constantly, and any risk managed. A mental health practitioner or nurse must be in physical attendance of the person at all times and make a record of observations made.
10E	Record of examination of restrained person and possible extension of bodily restraint Within 30 minutes, if the restraint is still being used, a Medical Practitioner must examine the patient and confirm whether the restraint should continue or stop. If restraint >30 minutes the restraint can be extended further in incremental periods of 30 minutes, but only following examination by a Medical Practitioner (MHA2014 s.234)
10F	Variation of bodily restraint order A medical practitioner or mental health practitioner can make an

	order to vary the physical restraints by shortening the period of restraint that is authorised for use.
10G	Revocation or expiry of bodily restraint order At any time during the restraint a Medical Practitioner or Mental Health Practitioner or person in charge of the ward may revoke the restraint order.
10H	Review of bodily restraint order by psychiatrist If the restraint duration exceeds 6 hours a psychiatrist must review the use of bodily restraint of the person and record the results [MHA2014 s.235(5)].
10I	Record of post restraint examination Within 6 hours of a restraining ending, a medical officer should conduct a physical examination of the person

8. Seclusion

- Seclusion applies to any patient receiving care regardless of their status under the MHA 2014 e.g. voluntary, referred or involuntary.
- Seclusion should always be the last intervention attempted. Initially attempt de-escalation, problem solving and other methods of managing the patient such as offering medication or use of a sensory room or other appropriate area for time out.
- A medical practitioner may make an order extending an existing seclusion order for a further period not exceeding two hours¹².
- Where a patient is in seclusion for greater than 24 hours care must be escalated to the Mental Health Advocacy Service (MHAS) and the Chief Psychiatrist at regular intervals.

8.1. Patient risk minimisation strategies

Seclusion can be a very distressing event and patients may attempt to harm themselves when in seclusion. To reduce risk:

- explain to the patient the method for calling for attention.
- ensure the patient is searched for articles they may use to self-harm and remove them.
- refer safety principles for restraint listed in Section 7.3 when appropriate.

- the seclusion room/area must meet the requirements listed in the [Chief Psychiatrists Standards for Authorisation of Hospitals under the Mental Health Act 2014](#) including but not limited to:
 - enabling two-way communication (intercom) with the patient when in seclusion
 - limited furnishings including a tear proof mattress which is to be unable to be utilised as a barricade or be turned on its side and stood on.
 - tear proof bed clothes
 - externally controlled air-conditioning and/or heating enabling those observing the patient to monitor the room temperature
 - being locked when not in use.

8.2. Patient observation and management

- Staff will attempt to explain the rationale for seclusion to the patient and assure them it is in their best interest and their safety will be maintained.
- The patient must be taken to the seclusion room by API 3 trained staff and not less than four staff members.
- Patients must be provided with appropriate clothing, enough food and drink and access to toilet facilities.
- A clock should be visible to the patient from within the room.
- Patients must not be left naked in seclusion and must be left with a means of keeping warm.
- Whilst in seclusion the patient must be observed every 15 minutes by a registered nurse or enrolled nurse⁶. The observations may be performed from outside the room but must be via direct observation not closed-circuit television.
- Where the patient cannot be monitored from outside the room the observation will be performed by two or more staff entering the room.
- Consider the gender of the staff member carrying out the ongoing observations. This decision should be informed by the patient's trauma history.
- Observations will include but are not limited to:
 - consciousness/ breathing/respiratory rate
 - pallor/colour
 - expressed aggression/agitation.
 - potential/threatening/actual harm.

- A medical practitioner must examine the patient at least every two hours and more frequently if clinically indicated and within six hours of release to ensure there are no complications.

8.3. Discontinuation of seclusion.

- As soon as the patient is clinically appropriate to leave seclusion they should be allowed to do so.
- Opening a door for toilet/food breaks or reviews does not constitute the end of a period of seclusion. The seclusion period is ended only when the patient is allowed free and unrestricted access to the normal ward environment.
- Seclusion will be discontinued as soon as the patient:
 - shows any sign of physical deterioration
- Seclusion also ends when the patient is asleep, and the door is opened allowing the patient to leave as desired.
- The decision to end seclusion will be made by two clinical staff members. The consumer must be informed that they are free to leave the room.
- Following seclusion continued close observation and contact with staff should be maintained until it is no longer clinically required.
- Efforts will be made to re-engage the patient and re-establish a therapeutic rapport on termination of seclusion and the seclusion episode will be documented in the patient's medical record.

8.1. MHA 2014 obligations

A copy of all MHA 2014 forms completed will be provided to the CP. Refer to Table 2 MHA 2014 obligations: Seclusion for details.

Table 2. MHA 2014 obligations: Seclusion.

MHA 2014 form	MHA 2014 obligation
11A	Record of oral authorisation of seclusion Seclusion can be authorised by a medical practitioner, mental health practitioner or the person in charge of ward is satisfied that: • The person needs to be secluded to prevent the person from: ○ Physically injuring themselves or another person; ○ Or persistently causing serious damage to property; and • There is no less restrictive way of preventing injury or damage

11B	Written seclusion orders Seclusion with a verbal order must be confirmed with a written seclusion order within 2 hours or the person released from seclusion. A, or person in charge of a ward may complete a written authorisation.
11C	Record of informing the medical practitioner or treating psychiatrist of seclusion order The mental health practitioner or person in charge of the ward must inform the: • medical practitioner as soon as practicable or with sufficient time to allow examination within 2 hours of the person being secluded. • treating psychiatrist as soon as practicable or with sufficient time to allow examination within 2 hours of the person being secluded.
11D	Record of observations made of secluded person. A mental health practitioner or a nurse must observe the patient at a minimum of every 15 minutes, more often if indicated e.g. if intramuscular PRN given and include any interventions e.g. hydration / toileting.
11E	Record of examination of secluded person and possible extension of seclusion The medical practitioner must examine the patient within 2 hours of being secluded (s.214) and at least every 2 hours thereafter (s.222) and more frequently if the patient's condition indicates more medical intervention. A medical practitioner may extend a seclusion order for a further period (not exceeding 2 hours) only if immediately before doing so the medical practitioner examines the person.
11F	Revocation or expiry of seclusion A medical practitioner, mental health practitioner or the person in charge of a ward at an authorised hospital may make an order revoking a seclusion order (s.219) or

	A medical practitioner, or mental health practitioner must as soon as practicable after a seclusion order expires make a record of the seclusion order expiring.
11G	Record of post seclusion examination Whenever a patient is released from seclusion the person in charge of the ward must ensure: • that the patient is examined by a medical practitioner within 6 hours after the patient is released from the seclusion; or • if the patient is to be released or discharged, or wants to leave the hospital against medical advice, the patient is to be offered an examination by a medical practitioner before the patient is released, discharged or leaves [MHA2014 s.223(2)]

9. Post restraint and/or seclusion

- Appropriate support will be provided following restrictive practice to enable all parties involved to discuss what has occurred, and to consider strategies to reduce the need for restrictive practices in the future.
- An episode of restrictive practice can be traumatic. Staff will acknowledge to the patient the elements of trauma in this and past experiences and take necessary steps to manage the consequences.
- The patient must not be coerced into a discussion. They will be informed that the episode can be discussed at a future stage if desired. The discussion will be documented on the Post Seclusion Patient Interview form.
- All patient support should be conducted by staff experienced and/or skilled in this area.
- If willing and appropriate to participate, the patient and PSP, family /significant other should be involved in this review.
- Following an episode of restraint and/or seclusion care will be taken to reintegrate the patient in a way that protects their safety and wellbeing and preserves their dignity and self-esteem. The patient will be informed of the wards activities and escorted as appropriate.
- Following the episode of restraint and/or seclusion the patient's TSDP must be reviewed with the patient and updated. The patient will be provided with an updated copy of their plan for their approval and signature.
- A post incident debriefing session is to be offered where appropriate to all staff immediately after an episode of restraint and/or seclusion.

- All persons involved in restraint and/or seclusion episodes will be offered individual counselling via the Employee Assistance Program or the FSFHG Staff Psychological Wellbeing and Support Program where appropriate.

10. Executive review of restraint and/or seclusion

- Executive review of restraint and seclusion episodes is a key restraint minimisation strategy.¹³
- In line with the Chief Psychiatrists Standards for Clinical Care the Mental Health Service will conduct a weekly Executive Review of all restraint and/or seclusion episodes. The review:
 - will focus on collaborative reduction of restrictive practice.
 - is not a blaming process.
 - will include staff involved in the episode wherever possible.
 - will include peer worker or individual with lived experience and/or family /significant other representative and aboriginal mental health liaison where possible.
- The review will be documented on an 'Executive Review of Restrictive Practices' form.

11. Compliance/performance monitoring

- MHA 2014 Restraint and Seclusion forms are reported to the Chief Psychiatrist for monitoring and publication.
- Restraint and seclusion data and trends are monitored monthly to the Service 5 Safety Quality and Risk Committee.
- Behaviour incident data and trends are monitored monthly at the Service 5 Safety Quality Risk Committee.
- Service 5 completes an executive review of all episodes of restraint and/or seclusion.
- The Mental Health Service will publish local de-identified seclusion and restraint data quarterly within the Service. This data will be made available to staff, patients and the general public and forwarded to the Chief Psychiatrist.
- API compliance rates will be monitored by line managers.

12. Related policy documents

- Chief Psychiatrist, Government of WA:
 - [Policy for mandatory reporting of notifiable incidents to the Chief Psychiatrist](#)
- Department of Health WA
 - [Clinical Incident Management Policy 2019](#)
 - [Clinical Incident Management Guideline 2019](#)
 - [Consent to Treatment Policy MMP 0175/22](#)
 - [Clinical Handover Policy MP 0095/18](#)
 - [Recognising and Responding to Acute Deterioration Policy](#)
 - [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
 - [Complaints Management Policy](#)
- SMHS:
 - [Workplace aggression and violence policy](#) SMHS: 127
 - [Workplace aggression and violence procedure](#)
 - [Employee assistance program](#)
 - [Rights of patients in mental health](#)
 - [Rights of carers and other personal support persons](#)
 - [Patient search and identification of dangerous items](#)
- FSFHG
 - Anticipating and reducing the risk of unpredictable behaviours (FSFH-MENH-POL-0027)
 - Clinical documentation (FSFH-HW-POL-0039)
 - Patient Observation - Specialising and Close Observations (FSFH-MENH-POL-0023)
 - Agitation, arousal in Mental Health inpatients: Medication management (FSFH-MENH-GUI-0004)
 - Observation and Response Chart (FSFH-HW-POL-0032)
 - Sensory rooms and sensory equipment; safe and therapeutic use (FSFH-MENH-GUI-0006)
 - Vicarious trauma (FSFH-MENH0GUI-01)
 - Code Black in the Mental Health Unit (FSH-MENH-POL-0014)

- FH Duress Alarms and Response (FSFH-MENH-PRO-0005)
- FSH - Code Blue
- Fremantle Hospital Code Blue – Medical Emergency
- Zuclopenthixol acetate (Clopixol Acuphase®) Specialised Drug Guideline (FSFH-PHARM-GUI-0010)
- Fixed Tray Table (FSFH-MENH-POL-0029)

13. Related standards

NSQHS Standards:

- Clinical Governance
- Partnering with Consumers
- Medication Safety
- Comprehensive Care
- Communicating for Safety
- Recognising and Responding to Acute Deterioration

Chief Psychiatrist's Standards for Clinical Care:

- Assessment
- Consumer and Carer Involvement in Care
- Seclusion and Bodily Restraint

14. References

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15. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision Due
1	03/2020	Clinical Nurse Specialist, Clinical Improvement	Service 5 SQR Committee	FSFHG Policy Committee	03/2022
2	05/2022	Clinical Nurse Specialist, Clinical Improvement	NMPGC	S5 SQR Committee	05/2024
3	05/2024	Clinical Nurse Specialist, Clinical Improvement	Feedback incorporated from the Chief Psychiatrist.	S5 SQR Committee	08/2026
3.1 Draft	07/2024	S5 SQR Lead	Minor Update, No change to review date	S5 SQR Committee	08/2026

16. Appendices

16.1. Appendix 1: Patient risk factors for physical restraint

Risk factor	Rationale
Obesity, large abdominal girth	High risk of positional asphyxia if restrained in prone position as abdomen may restrict movement of the diaphragm
Alcohol affected	- Respiratory depressant - Increased risk of aspiration
Prescribed or non-prescribed benzodiazepines	Respiratory depressant
Stimulant affected	- Potential for prolonged struggle - Physiological stress
Serious mental illness including mania/psychosis	- Potential for prolonged struggle - Physiological stress - Pharmacology: benzodiazepines, anti-psychotics with potential for increased QT interval and cardiac arrhythmias
Sickle cell anaemia	Exercise related collapse, local hypoxia creating sickling
Older adult	Musculoskeletal injury
Youth	Physical immaturity
Underlying cardiovascular or respiratory disorder	Exacerbation of symptoms
History of trauma	Exacerbation/ re-experience of trauma
Pregnancy	Potential for aortic compression by the uterus: position in left lateral position

16.2. Appendix 2: Team Roles and Responsibilities

Role	Primary responsibilities
Lead Clinician API No1 Clinician	• Support and protect the head and neck if required without restriction of movement causing adverse risks to the patient. • Maintain verbal communication with the patient, de-escalate where possible. • Communication with and instruction of the response team • Monitor interventions of other team members • Monitor airway, breathing circulation. • Communicate observations to remainder of team for decision making purposes
Incident Coordinator	• Allocation of response team members • Communication with all response team members • Maintain the safety of the environment and all people present. • Recording and documentation of restraint episode – or delegate • Communicate to Lead Clinician time frames the patient has been restrained in high-risk positions. • Post restraint debrief with the response team members. • Completion of appropriate MHA 2014 forms
Team members	• Follow Lead Clinician instructions. • Airway management • Assessment of patient's clinical condition and response to restraint – provide feedback to Lead Clinician • Restraint of arms • Restraint of legs • Administration of medications as prescribed

16.3. Appendix 3: Physical restraint positions and specific risks

Restraint Position	Risks	Risk reduction strategies
Prone (face down)	• Sudden unexpected death • May restrict the ability to breathe lessening the supply of oxygen to meet the body's needs • Should only be used as an intervention of last resort	• Continual patient monitoring and communication • No pressure placed on the head, neck, back • Patient not to be placed in prone maximal restraint position (hogtied) • Should be limited to the minimum amount of time⁸ necessary to change position preferably kneeling or standing i.e. < 3 minutes
Supine (face up)	• Choking or inhalation of vomit and aspiration • Risk of asphyxia, fatal cardiac arrhythmia or respiratory distress	• Keep patient's head or bed elevated to reduce aspiration risk
Seated	• Where the patient is leaning forward may increase risk of harm or death during prolonged restraint. Risk further increased if a higher body mass index • Where the patient's arms are wrapped around the front of the body and held from behind can increase risk of asphyxia	• Patient to be seated upright • Avoid hyperflexion of the spine • Do not wrap patient's arms around the front of their body